



Gavin Newsom, Governor
State of California
Health and Human Services Agency
DEPARTMENT OF MANAGED HEALTH CARE
980 9th Street, Suite 500
Sacramento, CA 95814
Phone: 916-324-8176 | Fax: 916-255-5241
www.DMHC.ca.gov

May 1, 2026

VIA ELECTRONIC MAIL

Karen Choate
Blue Cross of California
1121 L Street, Suite 500
Sacramento, CA 95814

Re: Approval with Conditions
Proposed Contract Termination with Valley Children's Hospital
Block Transfer Filing No. 2026-HOSP-294
Filed on March 18, 2026

Dear Ms. Choate,

The Department of Managed Health Care (Department) has reviewed the information in the above-referenced Block Transfer Filing (Filing) submitted by Blue Cross of California (Plan) for compliance with the requirements of the Knox-Keene Health Care Service Plan Act of 1975, as amended (Cal. Health & Safety Code § 1340 *et seq.*) and the regulations promulgated thereunder (Cal. Code Regs., Tit. 28, § 1000 *et seq.*).

In the event of contract termination, the Filing proposes to remove Valley Children's Hospital (Terminating Hospital) from the Plan's hospital network. The effective date of contract termination between the Terminating Hospital and the Plan (Termination Date) is currently July 1, 2026.

The Plan proposes to redirect affected enrollees to the following Alternate Hospitals:

- CHILDREN'S HOSPITAL OF LOS ANGELES
- CLOVIS COMMUNITY MEDICAL CENTER
- FRESNO SURGICAL HOSPITAL
- LUCILE PACKARD CHILDREN'S HOSPITAL STANFORD
- ST. AGNES MEDICAL CENTER
- UCSF BENIOFF CHILDREN'S HOSPITAL OAKLAND

Based on the Plan's representations and information provided through the review process, the Department hereby **approves** the Plan's Filing, subject to the following:

Protecting the Health Care Rights of More Than 30.2 Million Californians
Contact the DMHC Help Center at 1-888-466-2219 or www.DMHC.ca.gov

1. If the contract terminates, the Plan will allow its contracted providers and provider groups to continue referring pediatric services to the Terminating Hospital after the Termination Date, and to continue to allow all affected enrollees to receive at the Terminating Hospital after the Termination Date:
 - a. any medically necessary services available at the Terminating Hospital, but not available or cannot be scheduled in a timely manner consistent with good professional practice at an Alternate Hospital or other in-network contracted hospital facility within a reasonable geographic distance, such medically necessary services include, but are not limited to, the following services, or services within the following specialties: Acute Neonatal, Acute Pediatric, Heart Center / Cardiology, Pediatric Surgery, Orthopedics, Neuroscience Center / Neurology, ECMO (NICU specialties/services), Neonatology / NICU, Maternal Fetal Center / Maternal Fetal Medicine, Gastroenterology, and Cancer & Blood Disorders / Hematology-Oncology or
 - b. any medically necessary services available at the Terminating Hospital where an in-network provider holds admitting privileges at the Terminating Hospital and is unable to admit enrollees to an Alternate Hospital or other in-network contracted hospital facility within a reasonable geographic distance in a timely manner consistent with good professional practice, or
 - c. any medical necessary services available at the Terminating Hospital which, due to insufficient capacity, an enrollee may not be able to access at an Alternate Hospital or other in-network contracted hospital facility within a reasonable geographic distance in a timely manner consistent with good professional practice.
2. For the Plan's affected enrollees accessing the Terminating Hospital under Condition 1, the Terminating Hospital will continue to be deemed a participating, in-network provider. For these affected enrollees accessing the Terminating Hospital, the copayment, deductible, percentage coinsurance or any other cost sharing components for care accessed under the aforementioned conditions shall be no greater than the financial responsibility the enrollee would have had if the enrollee received the services from the Terminating Hospital prior to the Termination Date.

If a dispute arises between the Terminating Hospital and the Plan concerning the reasonable and customary value of the provider's service, the Plan shall protect and hold the enrollee harmless for any amount above the enrollee's in-network copay, deductible, coinsurance or other cost sharing component.

3. For the Plan's affected enrollees accessing the Terminating Hospital under Condition 1, the Plan will directly pay to the Terminating Hospital, as applicable:
 - a. the amount required by any post-termination rates set forth in the contract between the Plan and the Terminating Hospital,

- b. the amount set forth in any negotiated letter of agreement or other negotiated arrangement between the Plan and the Terminating Hospital regarding the services, or
 - c. the reasonable and customary value of the hospital's services, less the enrollee's established in-network copay, deductible, coinsurance or other cost sharing component.
- 4. The Plan will not take any adverse action, financial or other, against the Terminating Hospital with regards to the services identified in these conditions (this does not preclude the Plan or Terminating Hospital from utilizing any dispute resolution mechanism that would otherwise be available). In addition, the Plan will not take any adverse action, financial or other, against any contracted provider for admitting affected enrollees to the Terminating Hospital in order to receive services required by these conditions.
- 5. With respect to these conditions, "deemed to be a participating in-network provider" does not apply to emergency services or post-stabilization services rendered to Plan members at the Terminating Hospital following the Termination Date, nor does it affect any existing statutory and/or regulatory payment obligations with respect to these services.
- 6. No later than five (5) calendar days after the Termination Date, the Plan shall provide written notice to each enrollee who has an upcoming procedure authorized or scheduled at the Terminating Hospital **within the next six months**, as well as those who had accessed the Terminating Hospital **at least once during the past year**, subject to any waivers granted by the Department.

In addition, no later than five (5) calendar days after the Termination Date, the Plan shall provide written notice to all PPO enrollees living within 15 miles of the terminating hospital. The Plan may send this notice electronically if enrollees elect to receive electronic notification.

The notices shall be consistent with the conditions of this Approval, including informing enrollees of the availability to access the Terminating Hospital at in-network benefit levels.

- 7. Within five (5) calendar days following the Department's issuance of this decision, the Plan shall submit proposed provider notices to the Department for approval. The Plan shall not distribute provider notices until it receives Department approval as to the form and content of the notices.
- 8. No later than ten (10) calendar days following the Termination Date, the Plan will notify, in writing, those contracted providers and provider groups that may be affected by the contract termination. The provider notices shall be consistent with the conditions of this Approval and include:

- a. the termination and effective date of termination,
 - b. the conditions of this Approval, and
 - c. the method by which providers may refer and admit enrollees to the Terminating Hospital such that enrollees may access the Terminating Hospital at in-network rates in accordance with the conditions of this Approval.
9. With respect to enrollees who are approved to receive Completion of Covered Services from the Terminating Hospital, the Plan shall ensure that the copay, deductible, percentage coinsurance or any other cost sharing component for such care is no greater than the financial responsibility the enrollee would have had if he or she received the services from the Terminating Hospital prior to the Termination Date. (See Cal. Health & Safety Code § 1373.96.).
10. After reviewing the Plan's Filing, the Department has determined that in the event of contract termination, the enrollees residing or working in the following ZIP Codes within the Plan's approved service area for at least one applicable network will no longer have access to a contracting or plan-operated primary care provider within 15 miles or 30 minutes:
 - 93614
 - 93638
 - 93667

The Plan shall file with the Department, no later than 60 days following the Termination Date, a Notice of Material Modification requesting alternative standards of accessibility for the affected portion of its service area, including the ZIP Codes listed above, if the Plan does not have a pending or Department approved alternative access filing which already addresses the distance from the enrollees' residence or workplace to the Alternate Hospital(s) in the affected service area. (See Cal. Health & Safety Code § 1352; Cal. Code Regs., Tit. 28, §§ 1300.51(d)(H)(i) and 1300.67.2.1.).

11. The Department's Conditional Approval of this Block Transfer Filing and the requirement that the Plan submit a Notice of Material Modification requesting alternative access standards for those ZIP Codes identified as outside of the 15/30 mileage/minute standard does not constitute approval (express or implied) of an alternative access standard. The Notice of Material Modification required to be submitted by the Plan under Condition 10, above, will be reviewed in accordance with requirements set forth in the Act and, to the extent that one or more ZIP Codes cannot be approved for an alternate access standard, the Plan will be required to take all steps necessary to ensure that its provider network (throughout the Plan's entire service area) offers access in accordance with all applicable statutory and regulatory requirements concerning network adequacy.

(See Cal. Health & Safety Code §§ 1371.31(a)(5), 1367(d)&(e), & 1375.9; Cal. Code Regs., Tit. 28, §§ 1300.51(d)(H)&(I), 1300.67.2, 1300.67.2.1, 1300.67.2.2.)

If the Plan cannot ensure that its provider network offers access in accordance with all applicable statutory and regulatory requirements concerning network adequacy for any of the applicable ZIP Codes that cannot be approved for an alternate access standard, the Department may require the Plan to withdraw those ZIP Codes from its service area for each affected network.

12. If the Plan reaches an agreement with the Terminating Hospital after the above enrollee transfer notices are sent, the Plan must promptly inform the Department and convey an additional enrollee notification to each affected enrollee, in accordance with the requirements set forth in Cal. Code Regs., Tit. 28, § 1300.67.1.3(f).
13. In the event that the effective date of removal of the Terminating Hospital from the Plan's network is delayed by **90** or more days, the Plan must request, and the Department must grant, an extension of this Approval for it to remain in effect.
14. If the Plan determines that information submitted in this Filing is materially incorrect, it will promptly notify the Department. The Plan must also promptly notify the Department of any changed circumstances or new information that is material to this Filing.
15. The Department reserves the right to revoke, modify or terminate this approval at any time based upon changed circumstances or new information that is material to the filing, or for non-compliance by the Plan with any of the Conditions listed above.

Please contact me if there are any questions regarding the above.

Sincerely,

Elizabeth Doyle

Elizabeth Doyle
Analyst II
Office of Plan Monitoring